STAR TELERADIOLOGY

SimonMed

Patient Name:		Patient ID:	[redacted]
Referring Physician:			
Procedure:	MRI LUMBAR WO	Date of Study:	15-Oct-2025 11:19 AM

EXAM: MRI LUMBAR WO

HISTORY: Pain after a motor vehicle accident 10/01/25.

TECHNIQUE: Axial and sagittal multi-weighted images of the lumbar spine were obtained.

COMPARISON: None.

FINDINGS:
CONUS: Terminates in normal position and demonstrates normal signal characteristics.
OSSEOUS STRUCTURES: Vertebral bodies are normal height without fractures.
ALIGNMENT: No vertebral body listhesis is present.
BONE MARROW: Normal bone marrow signal. No focal osseous lesion is seen.
PARASPINAL SOFT TISSUES: Paraspinal soft tissues are unremarkable.

T12-L1: No disc herniation, central canal, or foraminal stenosis.
L1-L2: No disc herniation, central canal, or foraminal stenosis. Joint effusion without underlying arthritis bilateral facet joint(s). This can be seen in the setting of a capsular sprain or instabilityseries 10,001 image 28.
L2-L3: No disc herniation, central canal, or foraminal stenosis. Joint effusion without underlying arthritis bilateral facet joint(s). This can be seen in the setting of a capsular sprain or instability, series 10,001 image 23, 22..
L3-L4: There is a 2.2 mm left foraminal/far lateral protrusion type disc herniation resulting in mild left foraminal stenosis, series 10,001 image 16, series 10,002 image 11. No central canal or right foraminal stenosis.
L4-L5: 2 mm left foraminal/extraforaminal protrusion type disc herniation, series 2001 image 11. This results in mild left foraminal stenosis, series 10,002 image 4.
L5-S1: Mild age-appropriate disc desiccation without disc height loss or degenerative endplate changes. There is a 5.3 mm left lateralizing protrusion type disc herniation impinging the thecal sac and resulting in mild central canal stenosis and resulting in moderate left foraminal stenosis impinging the exiting left L5 nerve root, series 10,002 image 12, series 10,001 image 8, series 2002 image 8. Mild right foraminal stenosis. This narrows the left subarticular recess impinging the traversing left S1 nerve root, series 2001 image 7.

There are no significant underlying degenerative changes. No significant disc desiccation or disc height loss. No facet arthropathy. No osteophyte formation.

IMPRESSION:
1. L5-S1 demonstrates a left lateralizing disc herniation impinging the exiting left L5 nerve root and traversing left S1 nerve root.
2. Left foraminal/extraforaminal disc herniations L3-L4 and L4-L5 as above.

STAR TELERADIOLOGY

SimonMed

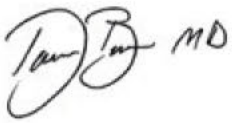
Patient Name:		Patient ID:	
Referring Physician:		Date of Birth:	[redacted]
Procedure:	MRI LUMBAR WO	Date of Study:	15-Oct-2025 11:19 AM

3. Joint effusion without underlying arthritis bilateral facet joint(s). This can be seen in the setting of a capsular sprain or instability.
4. Based on the imaging findings and within a reasonable degree of medical probability, the observed disc herniations may be temporally and causally related to a recent traumatic event rather than solely attributable to a chronic, degenerative process. Features that support a traumatic etiology may include the absence of significant endplate changes or large posterior osteophyte formation, presence of focal annular disruption, and morphology or location of the herniation that is atypical for age-related degeneration. Additionally, the lack of multilevel degenerative change or disc space narrowing at the affected level may further suggest that the pathology is more likely to be post-traumatic in nature. This interpretation should be considered in conjunction with the clinical history, mechanism of injury, and absence of prior comparable imaging for direct comparison.

The descriptions/nomenclature of any reported disc pathology, specifically with respect to the definitions of a disc bulge or disc herniation (protrusion type disc herniation and/or extruded disc herniation), is based on the recommendations from the following peer reviewed journal article: Lumbar Disc Nomenclature: version 2.0: Recommendations of the combined task forces of the North American Spine Society, the American Society of Spine Radiology, and the American Society of Neuroradiology. Spine J. 2014 Nov 1;14(11):2525-45. Any references to causation and permanency follow the guidelines established by the American Medical Association Guides to the Evaluation of Permanent Impairment, Sixth Edition, 2023. It should be noted that a normal MRI does not exclude certain pathologies that may be difficult to diagnose radiographically including certain pathologies involving small localized nerves and facet joints. A normal MRI does not exclude the possibility of certain abnormalities that may only be detected with a thorough physical examination and clinical history.

Electronically signed by:
Darren Buono, MD
10/19/2025 12:03 PM EDT

Electronically Signed: Date: 19-Oct-2025 12:03 PM



Darren Buono M.D.
Board Certified: American Board of Radiology
NPI: 1912004698

© 2025 Star Teleradiology. All rights reserved.
© 2025 Star Radiology. All rights reserved.
© 2025 Darren Buono, M.D. All rights reserved.

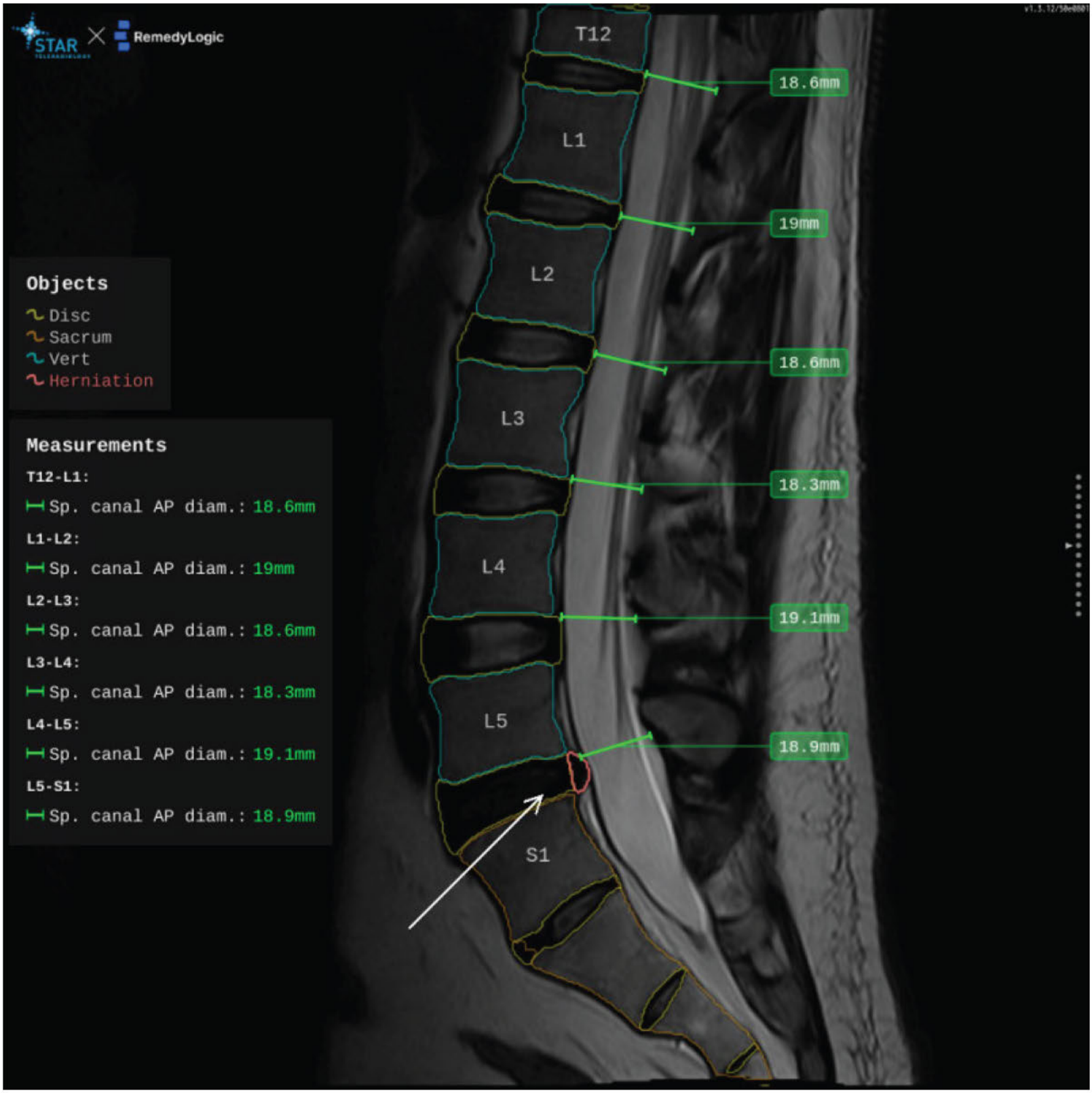
STAR TELERADIOLOGY

SimonMed

Patient Name:		Patient ID:	
Referring Physician:		Date of Birth:	[redacted]
Procedure:	MRI LUMBAR WO	Date of Study:	15-Oct-2025 11:19 AM

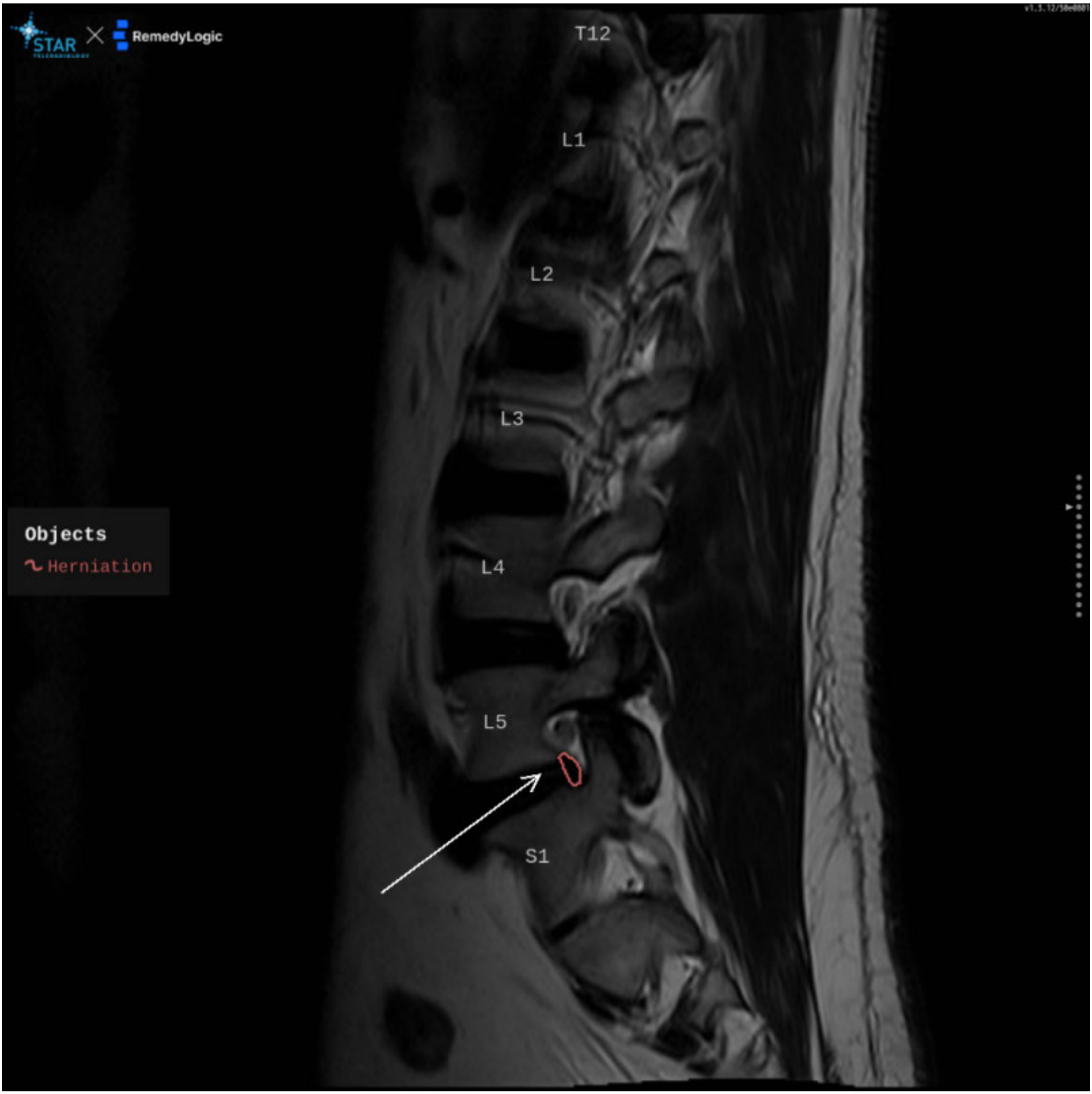
This material is protected under U.S. and international copyright laws. Unauthorized reproduction, distribution, or use of any portion of this document without express written consent from the copyright holder is strictly prohibited.

SimonMed



If you would like to speak to a radiologist regarding this report, please call: 1-855-723-8255

SimonMed



If you would like to speak to a radiologist regarding this report, please call: 1-855-723-8255

SimonMed



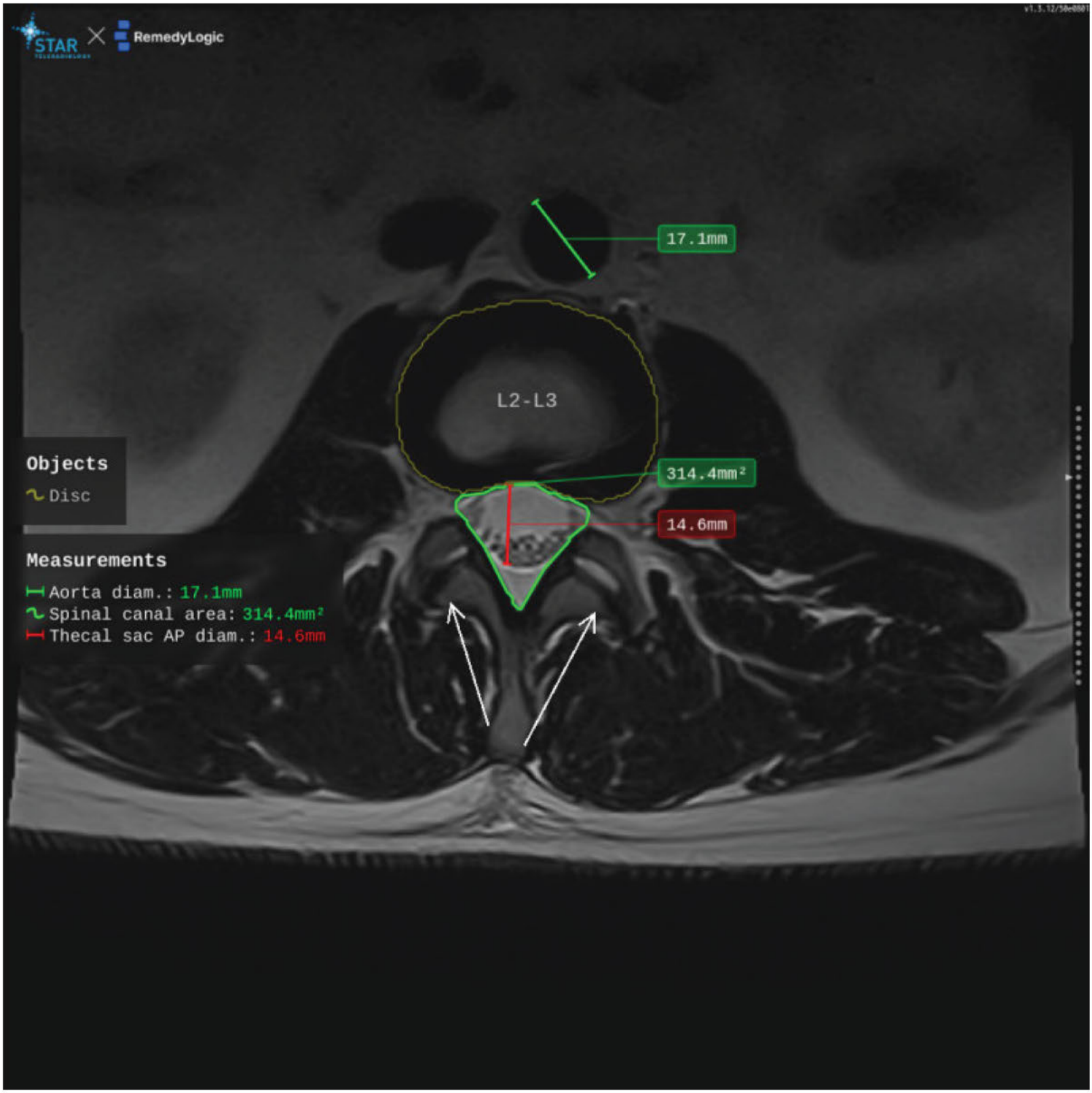
If you would like to speak to a radiologist regarding this report, please call: 1-855-723-8255

SimonMed



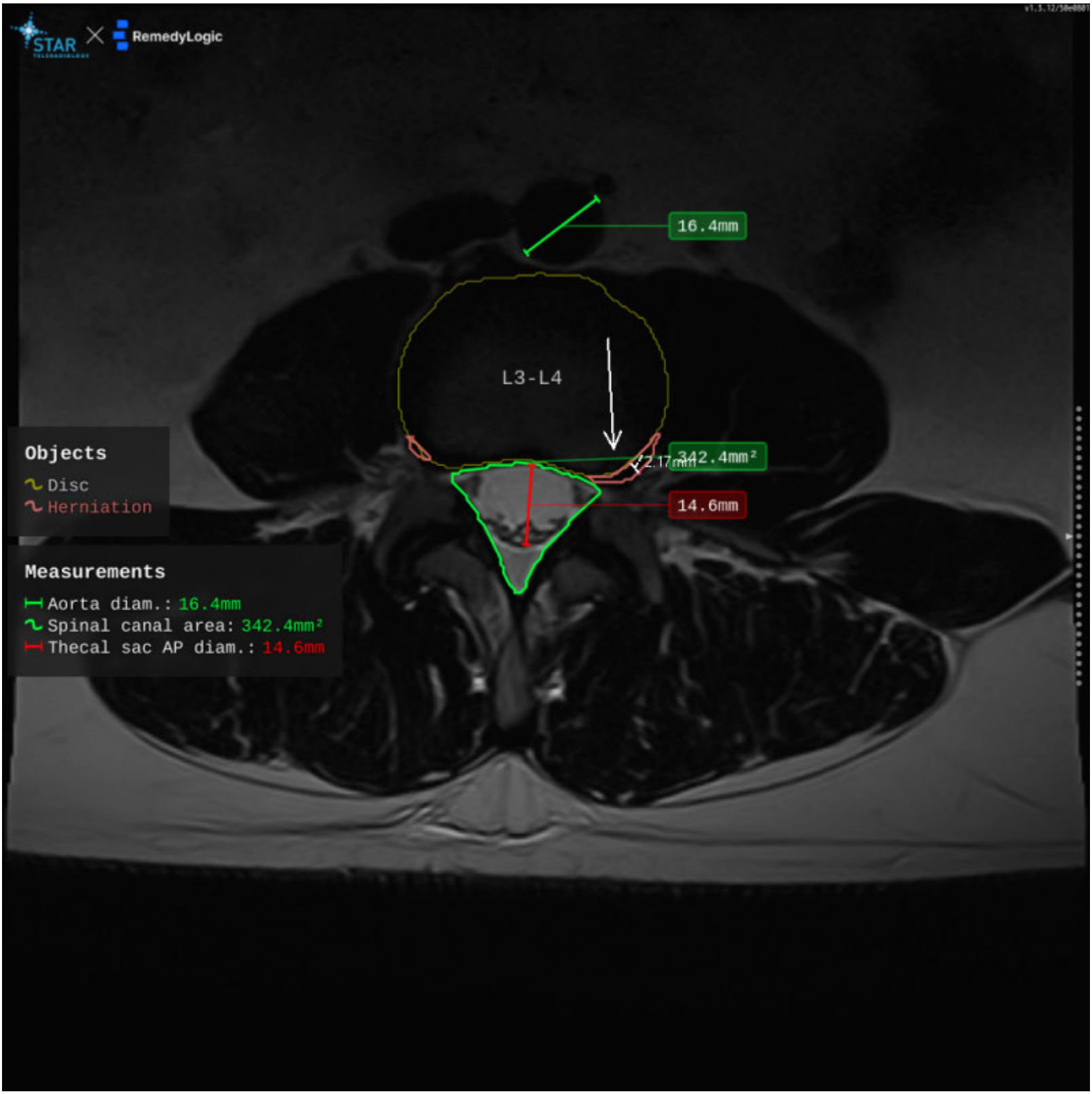
If you would like to speak to a radiologist regarding this report, please call: 1-855-723-8255

SimonMed



If you would like to speak to a radiologist regarding this report, please call: 1-855-723-8255

SimonMed



If you would like to speak to a radiologist regarding this report, please call: 1-855-723-8255

The EMG Center
Serving Central Pennsylvania:
Quarryville, Willow Street, Oxford, & Lebanon
Email: Referrals@TheEMGCenter.com
Phone: (223) 529-8049
Fax: (717) 464-7002

Patient:		**DOB:**	**Date Of Service:**
Height:	**Weight:**	**Sex:**	**Ref Phys:**

CHIEF COMPLAINTS / MEDICAL HISTORY: Pt presents with L sided LBP and L knee pain, with pain/paresthesias that radiate from her posterior hip to her posterior thigh/knee, posterolateral lower leg, and dorsal foot. Reports L knee pain has been chronic, and reports symptoms radiating down her leg have been ongoing "off and on" for the past 6 months. She denies any MOI. Denies experiencing any weakness in her L leg, and denies symptoms to R leg or B uppers. Reports symptoms are worse with standing/walking or sitting for longer periods of time. PMHx includes L knee arthroscopy. Denies DM, kidney, thyroid, autoimmune disease, cancer, exposure to toxins, or known family hx of neurological disease.

PHYSICAL EXAMINATION: Sensation to light touch is normal L1-S2 dermatomes B. Patellar and Achilles tendon reflexes are normal (2+) B. Slump test (-) B, PSLR tests (-) B. Lumbar extension AROM is limited and painful. Tinel's test fib head (+) L and (-) R, (-) med ankle B. Clonus (-) B. MMT: 5/5 throughout L1-S2 myotomes B. Mild edema observed L knee. Foot temp recorded and maintained at 30C or above throughout exam.

ELECTRONEUROMYOGRAPHIC STUDY:

Nerve Conduction:

- B sural sensory and L tibial motor are WNL. B tibial H-reflex are WNL.
- L superficial peroneal sensory and B peroneal motor are WNL.

Electromyography:

- L TA and lower lumbar paraspinals demonstrate muscle membrane instability. L TA and glute med demonstrate larger than normal muscle motor units.
- All other muscles sampled, L lower extremity L2-S1, are WNL in insertional, resting, MUAP morphology, and recruitment activities.

IMPRESSIONS: This is an abnormal study.

1. **There is electrophysiologic evidence suggestive of an active / ongoing lumbar radiculopathy affecting the ventral and dorsal rami of the L5 nerve root level on the left side.**

2. **There is no electrophysiologic evidence suggestive of any focal mononeuropathy, lumbosacral plexopathy, generalized polyneuropathy, or myopathy at this time.**

Adam Soltys

Adam Soltys, PT, DPT, ECS, COMT
Board Certified Electrophysiologist

Nerve Conduction Studies

Motor Summary Table

Site	NR	Onset (ms)	Norm Onset (ms)	O-P Amp (mV)	Norm O-P Amp	Neg Dur (ms)	Full Dur (ms)	Neg Area (mV·ms)	Site1	Site2	Delta-0 (ms)	Dist (cm)	Vel (m/s)	Norm Vel (m/s)
Left Peroneal Motor (Ext Dig Brev)														
Ankle		4.7	<6.5	6.4	>1.3	6.56	20.47	21.29	Ankle	Ext Dig Brev	4.7	8.0		
B Fib		11.4		6.0		6.72	19.06	20.56	B Fib	Ankle	6.7	32.0	48	>38.0
Poplt		12.8		5.8		6.72	18.44	20.64	Poplt	B Fib	1.4	6.0	43	>42.0
Right Peroneal Motor (Ext Dig Brev)														
Ankle		4.5	<6.5	7.2	>1.3	5.63	14.69	19.54	Ankle	Ext Dig Brev	4.5	8.0		
B Fib		11.6		6.7		7.34	15.16	18.96	B Fib	Ankle	7.1	32.0	45	>38.0
Poplt		13.0		6.5		6.41	15.00	17.93	Poplt	B Fib	1.4	6.0	43	>42.0
Left Tibial Motor (Abd Hall Brev)														
Ankle		3.9	<6.1	10.7	>4.4	5.78	14.53	29.14	Ankle	Abd Hall Brev	3.9	8.0		
Poplit		11.9		7.0		6.56	15.31	23.24	Poplit	Ankle	8.0	37.0	46	>39.0

Sensory Summary Table

Site	NR	Peak (ms)	Norm Peak (ms)	O-P Amp (µV)	Norm O-P Amp	Site1	Site2	Delta-P (ms)	Dist (cm)	Vel (m/s)	Norm Vel (m/s)
Left Sup Peron Sensory (Ant Lat Mall)											
Low Leg		3.8	<4.2	8.7	>3.0	Low Leg	Ant Lat Mall	3.8	14.0	37	>33.0
Left Sural Sensory (Lat Mall)											
Calf		3.0	<4.5	8.7	>4.0	Calf	Lat Mall	3.0	14.0	47	>31.0
Right Sural Sensory (Lat Mall)											
Calf		3.4	<4.5	6.8	>4.0	Calf	Lat Mall	3.4	14.0	41	>31.0

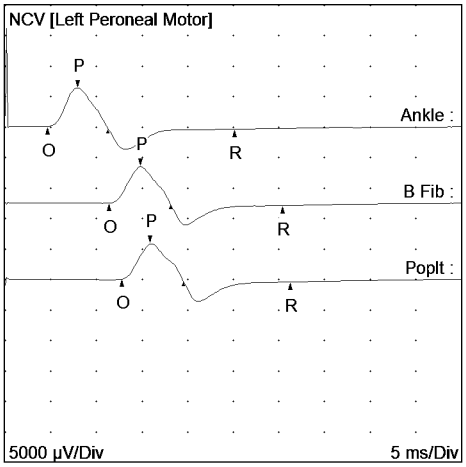
H Reflex Studies

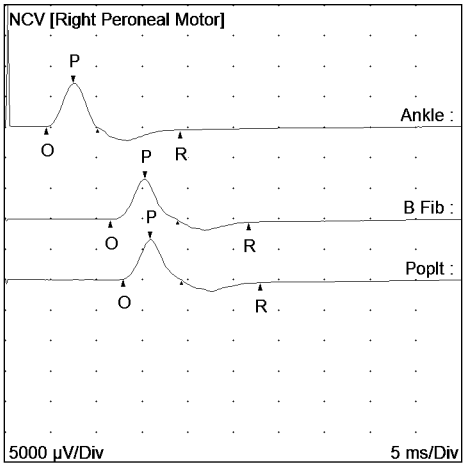
NR	H-Lat (ms)	Lat Norm (ms)	L-R H-Lat (ms)	L-R Lat Norm	Leg Length (cm)
Left Tibial (Gastroc)					
	30.52	<32	0.67	<2.0	37.00
Right Tibial (Gastroc)					
	29.85	<32	0.67	<2.0	37.00

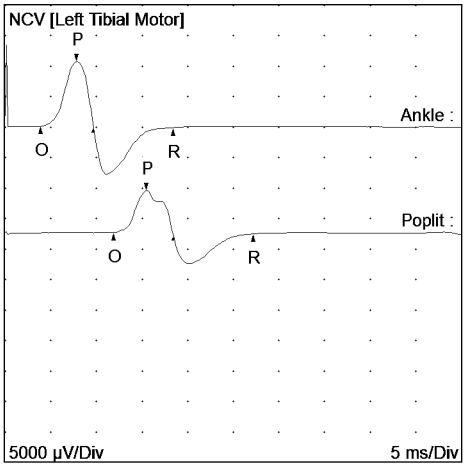
EMG

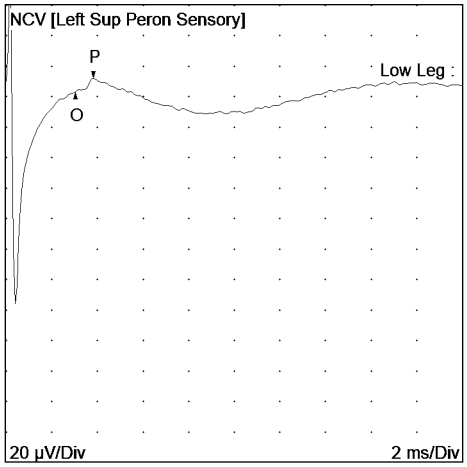
Side	Muscle	Nerve	Root	Ins Act	Fibs	Psw	Amp	Dur	Poly	Recrt	Int Pat	Comment
Left	Med Gastroc	Tibial	S1-2	Nml	0	0	Nml	Nml	Nml	Nml	Complete	
Left	Peroneus Long	Sup Br Peron	L5-S1	Nml	0	0	Nml	Nml	Nml	Nml	Complete	
Left	GluteusMed	SupGluteal	L5-S1	Nml	0	0	Nml	**Inc**	Nml	Nml	Complete	
Left	AntTibialis	Dp Br Peron	L4-5	**Inc**	**1+**	0	**Inc**	**Inc**	Nml	Nml	Complete	
Left	VastusMed	Femoral	L2-4	Nml	0	0	Nml	Nml	Nml	Nml	Complete	
Left	LumbParasp Lower	Rami		**Inc**	**1+**	0						CRD

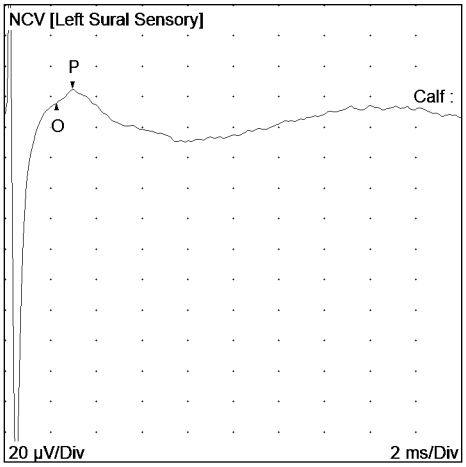
Waveforms:

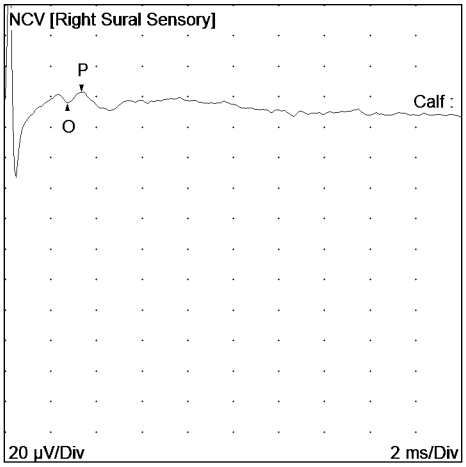


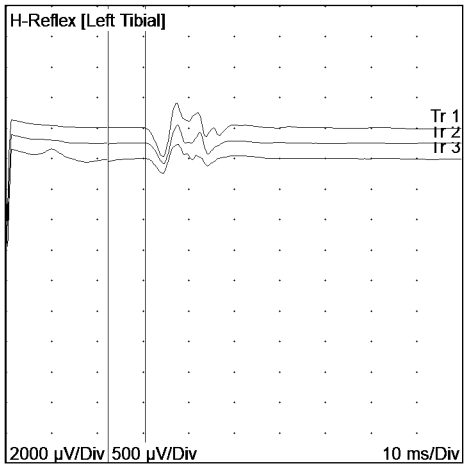


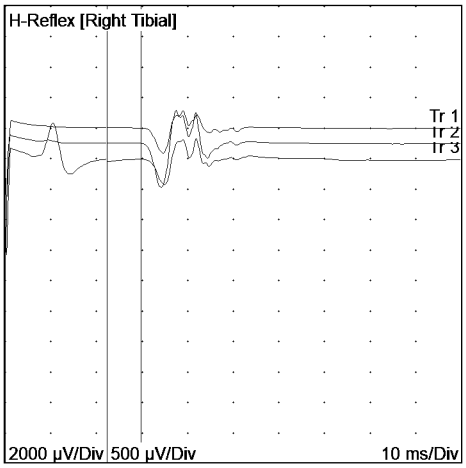


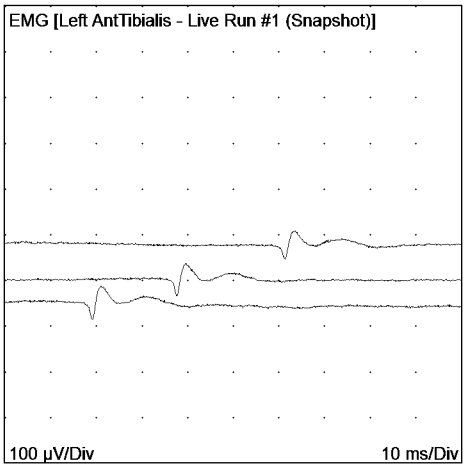

NCV [Left Sup Peron Sensory]
P
O
Low Leg :
20 µV/Div
2 ms/Div

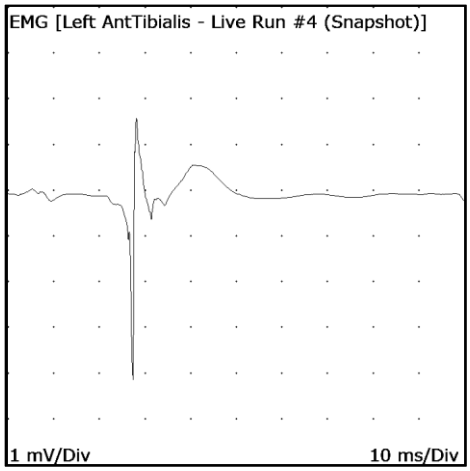

NCV [Left Sural Sensory]
P
O
Calf :
20 µV/Div
2 ms/Div

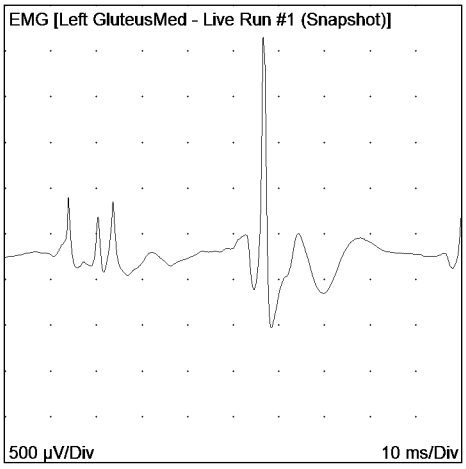

NCV [Right Sural Sensory]
P
O
Calf :
20 µV/Div
2 ms/Div

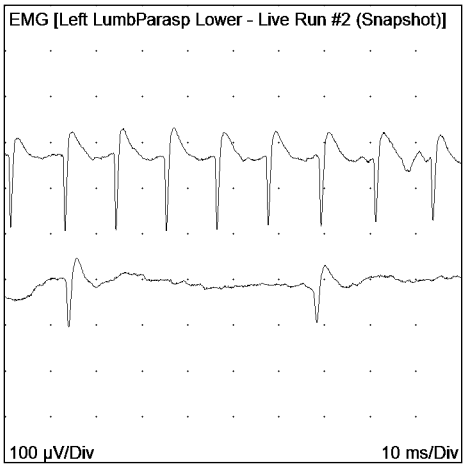

H-Reflex [Left Tibial]
Tr 1
Tr 2
Tr 3
2000 µV/Div
500 µV/Div
10 ms/Div


H-Reflex [Right Tibial]
Tr 1
Tr 2
Tr 3
2000 µV/Div
500 µV/Div
10 ms/Div


EMG [Left AntTibialis - Live Run #1 (Snapshot)]
100 µV/Div
10 ms/Div


EMG [Left AntTibialis - Live Run #4 (Snapshot)]
1 mV/Div
10 ms/Div


EMG [Left GluteusMed - Live Run #1 (Snapshot)]
500 µV/Div
10 ms/Div


EMG [Left LumbParasp Lower - Live Run #2 (Snapshot)]
100 µV/Div
10 ms/Div

	Units by Extremity		Description
	L	R	
95886 Upper			EMG Complete (5 or more muscles) each extremity; with or w/out paraspinals when performed with NCS
Lower	1		
95885 Upper			EMG Limited (<5 muscles) each extremity; with or w/out paraspinals when performed with NCS
Lower			
95907			Nerve conduction studies 1-2 tests
95908			Nerve conduction studies 3-4 tests
95909			Nerve conduction studies 5-6 tests
95910	1		Nerve conduction studies 7-8 tests
95911			Nerve conduction studies 9-10 tests
95912			Nerve conduction studies 11-12 tests
95913			Nerve conduction studies 13 tests

Diagnosis: M54.16, R20.2